TB infection, or develop into full-blown disease. In the time that Lokhande was struggling to find treatment for her XDR-TB, her younger sister contracted the disease too "since doctors failed to advise the family on how TB spreads at home". Data in the India TB Report bears this out. Around 64 per cent of TB-symptomatic individuals did not seek healthcare. Most commonly, they ignore symptoms (68 per cent) or don't recognise them to be those of TB (18 per cent), and self-medicate (12 per cent). Another 2 per cent cannot afford care. The DRS concluded that 56 per cent of the estimated MDR patients were undiagnosed and 64 per cent untreated.

DANGERS OF DRUG RESISTANCE

Social stigma is another reason why TB goes undiagnosed. Despite it breaking the class barrier, both perceptually as well as in terms of caseload, "TB remains a disease of the poor," says Blessina Kumar, CEO of the Global Coalition of TB Advocates, and a survivor herself. "And the burden is usually the patient's to bear alone." They either don't go for diagnosis out of fear or stop treatment midway rather than continuing the regimen for the requisite six months or more. This is one of the main reasons why the bacteria turns drug-resistant. It is also why the government chose to extend its earlier Directly Observed Therapy (DOT) programme, as part of which designated health workers ensured that TB patients took their prescribed drugs daily and varied them according to the healing prognosis.

There is also a problem in the health system itself. "Often," says public health expert Chapal Mehra, "testing is delayed because it is either never ordered or ordered very late. So, even people within the health system don't know if the patient has DR-TB and if they do, they don't know what drugs it is resistant to." It is an ordeal Aditi Mathur (*name changed*), 33, a homemaker from Mumbai, remembers all too well. "I felt like a human guinea pig for four years as doctors kept trying and failing to treat my disease. I had MDR-TB and because they didn't know what kind of resistance I had, they wasted the first two years by treating it with the incorrect drugs."

Making the DR-TB blow crueller still is the physical and financial burden of treatment. There are relatively fewer drugs to treat DR-TB, and new drugs are not widely available. The drugs are also far more toxic, causing side-effects that include gastritis, headache, peripheral neuropathy, depression, and, as in Lokhande's case, even hearing loss. Ashna Ashesh, a Patna-based lawyer and MDR-TB survivor, also found TB to be physically and emotionally consuming. "Depression, in a way, was also a kind of consumption, not of the tubercular variety that causes muscle wastage but consumption nevertheless in the sense of existential wastage," she says.



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DIPTENDU BHATTACHARYA | 37, Educationist, Kolkata

REALISING THE VALUE OF LIFE

Diptendu was diagnosed with MDR-TB in 2014, when he was still a student at IIT Kharagpur. The disease turned his life upside down. Initially, the doctors did not know that his TB was resistant to the first line of drugs. He spent days reading *Harry Potter* and listening to music, hopeful that he would be able to return to college soon. But that was not to be: after almost a year of treatment, the doctors realised that his treatment would have to be extended, and with far more toxic drugs. "On the day I took the first [medicine], I couldn't raise my hand beyond the shoulder level.... Soon, I had to buy the injections commercially. The costs made it difficult, but so did the anxiety of missing treatment," recalls Diptendu. He also suffered from severe side effects: "My ears were getting dry, my taste buds got funny, and my eyesight and handwriting went haywire." Reflecting on those years, Diptendu says TB has taught him to value life over material things. "It left me saner and wiser."